



Advancing Lanifibranor: Potential Best-in-Disease Oral Therapy for MASH

A DIFFERENTIATED ORAL ANTIFIBROTIC FOR MASH

Jefferies Global Healthcare Conference
June 3, 2026

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These statements include, but are not limited to, Inventiva's expectations regarding its ability to execute its new debt facility with BlackRock and Claret Capital Partners (the "Debt Financing Transaction"), agreement to repurchase certain of the EIB's existing warrants (the "EIB Transaction") and underwritten offering of ADSs (the "Equity Offering"), each announced on June 2, 2026 and subject to closing conditions (collectively, the "Combined Transaction"), in whole or in part and the timing thereof, including the timing of issuances of securities and receipt of proceeds in the Combined Transaction, Inventiva's ability to satisfy conditions to the subscription agreement with BlackRock and Claret Capital Partners and related debt financing documents (collectively, the "Issue Documents"), including any additional equity financings, any approval of Inventiva's shareholders required by the Combined Transaction, including the expected timing of any such required approval and the impacts of Inventiva's failure to obtain such approval, including with respect to waivers made by the EIB of certain anti-dilution and put option rights in connection with the EIB Transactions and the exercise ratio applicable to the EIB's warrants, the timing of and Inventiva's ability to draw down the committed amounts from BlackRock and Claret Capital Partners, the occurrence of an event of default under the Issue Documents, the potential exercise of warrants, including the T3 warrants issued by Inventiva on May 7, 2025, the expected timing, size and use of proceeds of the Debt Financing Transaction and the Equity Offering, forecasts and estimates with respect to Inventiva's current cash resources, and expected cash resources following the completion of the Combined Transaction, Inventiva's expectations with respect to ownership in its share capital by certain investors, Inventiva's capitalization following the completion of the Combined Transaction, Inventiva's NATIV3 Phase 3 clinical trial of lanifibranor in MASH, including the timing of clinical trial data releases and regulatory filings and future activities, expectations, plans, growth and prospects of Inventiva, and the absence of material adverse events. 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Given the risks and uncertainties, no representations are made as to the accuracy or fairness of such forward-looking statements, forecasts, and estimates. Furthermore, forward-looking statements, forecasts and estimates only speak as of the date of this presentation. Readers are cautioned not to place undue reliance on any of these forward-looking statements.

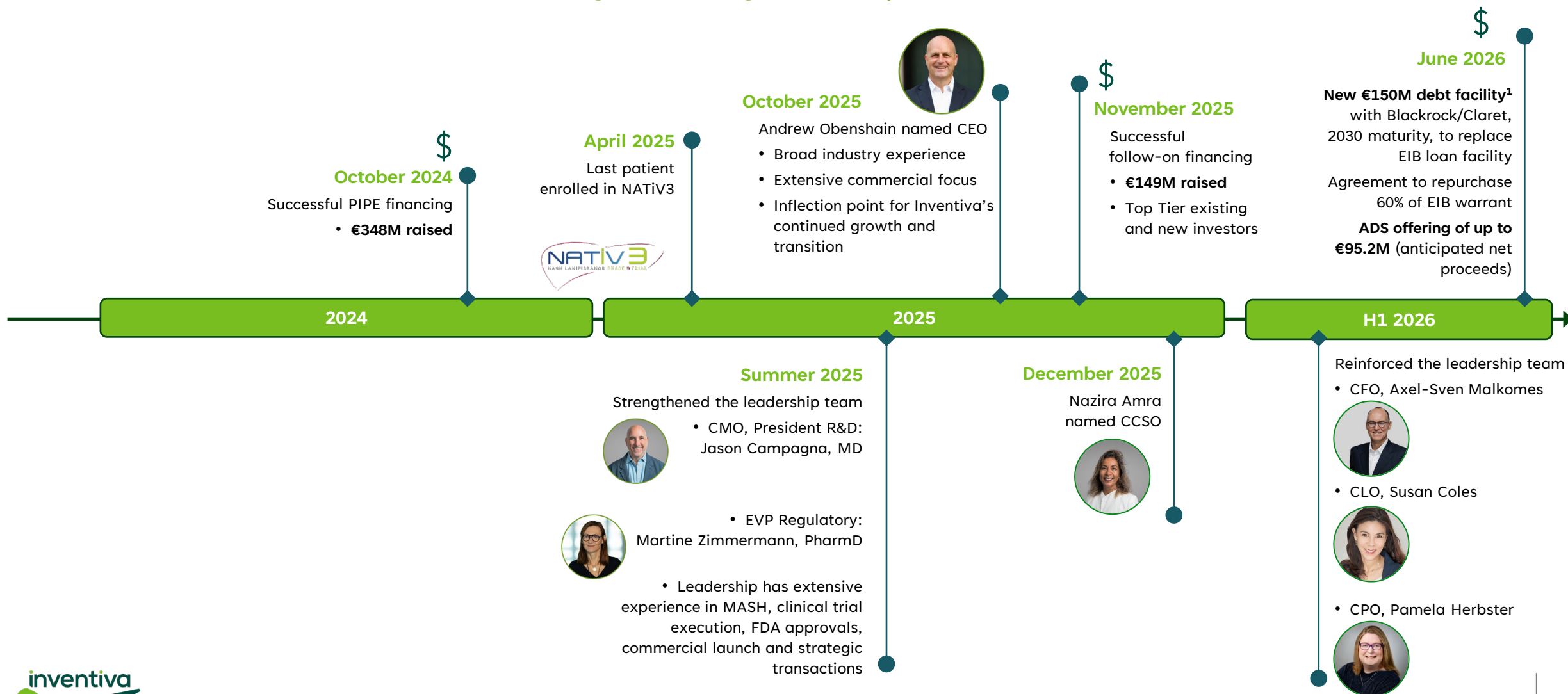
Please refer to the Universal Registration Document for the year ended December 31, 2025 filed with the Autorité des Marchés Financiers on April 8, 2026, and the Annual Report on Form 20-F for the year ended December 31, 2025 filed with the SEC on April 8, 2026 for other risks and uncertainties affecting Inventiva, including those described under the caption "Risk Factors", and in future filings with the SEC. Other risks and uncertainties of which Inventiva is not currently aware may also affect its forward-looking statements and may cause actual results and the timing of events to differ materially from those anticipated. All information in this presentation is as of the date of the release. Except as required by law, Inventiva has no intention and is under no obligation to update or review the forward-looking statements referred to above. Consequently, Inventiva accepts no liability for any consequences arising from the use of any of the above statements.

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Business Structured To Deliver Topline Results Expected in Q4 2026 and Prepare For Potential Launch

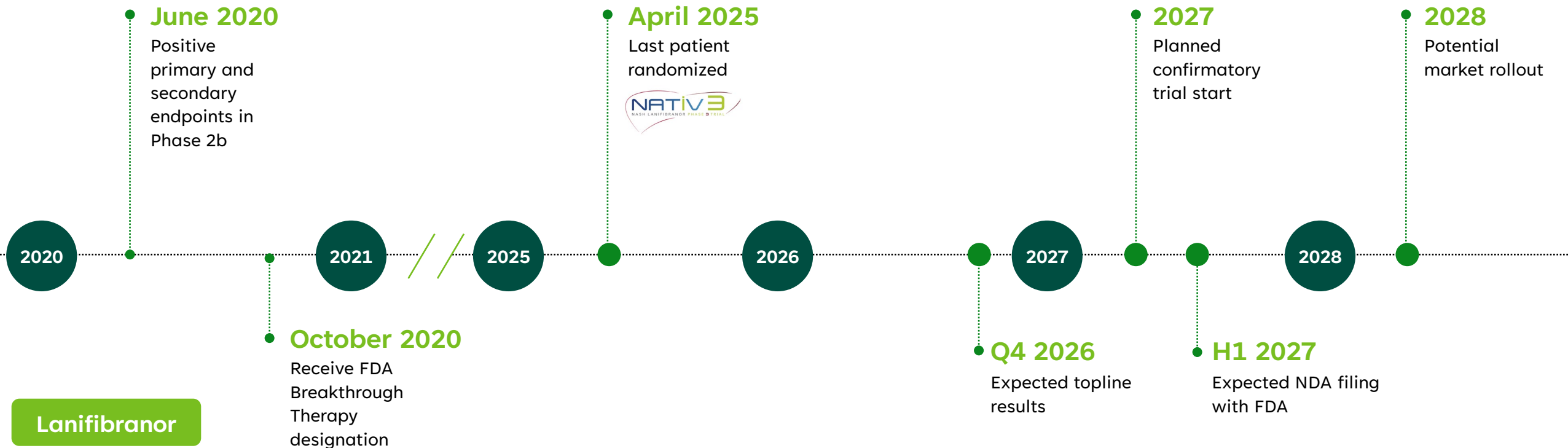
Clinical momentum, financial backing and strong leadership



1. New debt facility signed, subject to closing, for up to €130 million in committed tranches, subject to conditions, plus an additional uncommitted tranche of up to €20 million.

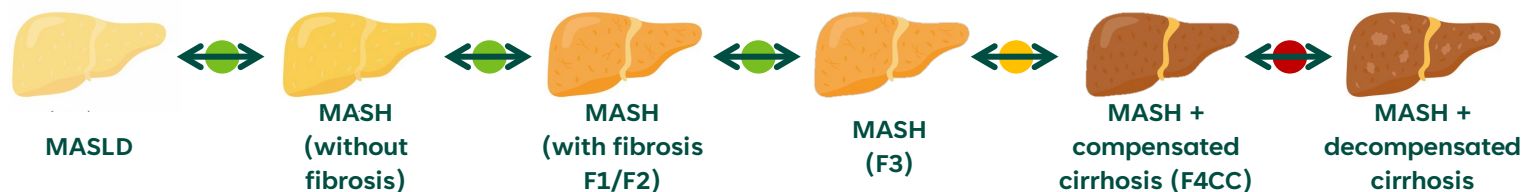
Topline Data Expected in Q4 2026, NDA Filing Expected in H1 2027

Lanifibranor has the potential to be the second oral liver-directed agent for the treatment of MASH if approved

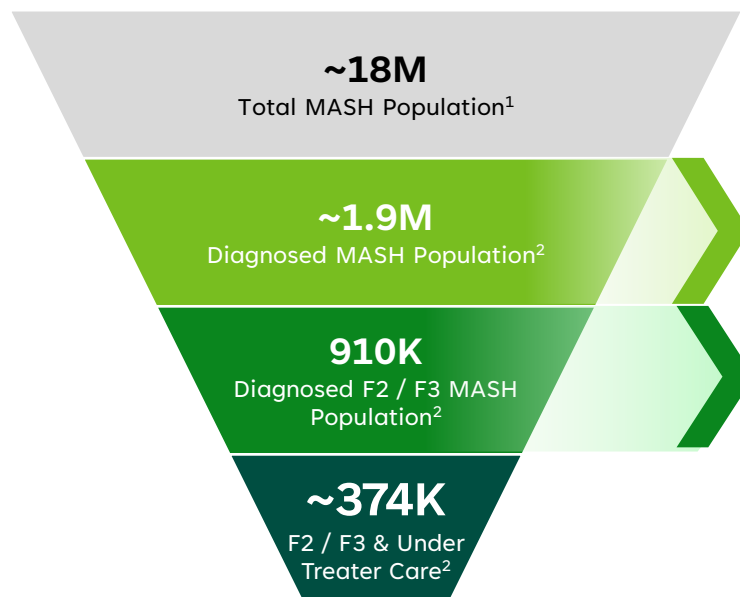


Underdiagnosed and Undertreated, With Rapidly Expanding Commercial Validation

A potential multi-billion-dollar opportunity for lanifibranor



TREATABLE MASH POPULATION



Only 10% of the overall patient population diagnosed

Clinically actionable patient population F2/F3

+25%

Compared to the 2024 estimate

+20%

Compared to the 2024 estimate

\$15B

MASH market expected to exceed \$15 billion by 2035.

Inventiva is well-positioned to potentially bring the best-in-disease oral drug to patients with progressive fibrotic MASH.

Lanifibranor's Profile In Key MASH Patient Segments, Based On Phase 2b Data – Subject To Phase 3 Confirmation

Potential best-fit patient groups for lanifibranor

	MASH F2 Fibrosis	MASH F3 Fibrosis
Non-T2D	Relevant competitive opportunity (~27%) Broad disease modification potential	Meaningful clinical fit (~12%) Significant fibrosis improvement in high-risk patients
T2D	Highly-competitive target segment (~36%) Fibrosis improvement + HbA1c reduction	Strong differentiation potential (~25%) Fibrosis improvement + HbA1c reduction



If approved, lanifibranor, due to its dual mechanism combining intra- and extra-hepatic activities, could address highly underserved MASH patient segments.



If Phase 3 confirms the Phase 2b profile, if approved, lanifibranor has the potential to offer a differentiated treatment option.

Balanced Pan-PPAR Designed to Address the Biology of MASH

A new chemical entity engineered to deliver
coordinated metabolic, anti-inflammatory and
antifibrotic effects

Rational Pan-PPAR Design Informed By Decades Of Biology

Lanifibranor: Low-potency, balanced PPAR- α / δ / γ activation designed to avoid receptor dominance



Potential first-in-class new chemical entity

Distinct from fibrates and thiazolidinediones (TZDs).



Differentiated co-activator fingerprint

Enable metabolic, anti-inflammatory, and antifibrotic benefits.



Balanced PPAR isoforms engagement

Avoid dominance of a single receptor and delivers coordinated metabolic, inflammatory, and antifibrotic activity.

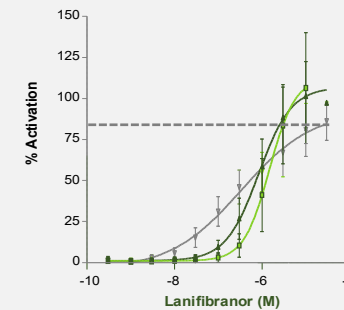


Balanced & low potency across all three isoforms

Engineered to mirror the safe potency range of pioglitazone and avoid the high γ -potency of rosiglitazone (~ 20x higher).

A new chemical entity (not a fibrate, not a TZD) with FDA breakthrough therapy & fast track designation

BALANCED RECEPTOR ACTIVATION PROFILE



SELECTIVE CO-FACTOR RECRUITMENT PROFILE



Lanifibranor has been designed at the physiologic sweet spot aiming for broad and best-in-class efficacy and a clean and durable safety profile



Engineered to maximize therapeutic benefit without transcriptional overload or high-potency risks



PPARs enable coordinated modulation of the metabolic, inflammatory, and fibrotic pathways at the core of MASH

Well Characterized and Differentiated Preclinical Safety Profile

Validated for long-term (≥ 6 months) and Phase 3 study



NO CARCINOGENIC FINDINGS

2-year rodent studies

Rat: No neoplastic changes or increased tumor types associated with single PPAR γ or dual PPAR α/γ agonists — including liver, adipose, bladder, renal, and skin

Mouse: No neoplastic changes of human relevance observed



FAVOURABLE TOLERABILITY PROFILE

12-month primate study

- No adverse clinical signs at any dose level tested
- No effects on body or heart weight; no haemodilution or creatinine increase
- ECG and clinical pathology investigations revealed no undesirable effects

Absence of class-defining toxicities — no evidence of:



Cardiac Toxicity



Renal Toxicity



Skeletal Muscle Toxicity



Bladder Findings



Proliferative Findings

**Clear differentiation vs
historical PPAR liabilities**

FDA Class Clinical Hold Lifted¹

Striking The PPAR Balance

PPARs enable coordinated modulation of the metabolic, inflammatory, and fibrotic pathways

LIVER

Steatosis (abnormal fat buildup)

PPAR- γ	↓ Fatty acid uptake
	↑ Fatty acid breakdown (catabolism)
	↓ Fat creation (lipogenesis)

Inflammation and Ballooning

PPAR- γ , PPAR- α , PPAR- δ	↓ NFkB-dependent gene activation
	↓ Inflammasome
	↓ Ballooning

Fibrosis

PPAR- γ , PPAR- α	↓ Stellate cell proliferation and activation
	↓ Collagen and fibronectin production

Vascular

PPAR- γ , PPAR- δ	↓ Portal pressure
	↓ LSEC capillarization
	↓ Intrahepatic vascular resistance

WHOLE BODY

Metabolism

PPAR- γ , PPAR- α , PPAR- δ	↓ HbA1c
	↓ Fasting Glucose
	↑ HDL-C
	↓ HOMA-IR
	↓ Fasting Insulin
	↓ Triglycerides
	↑ Adiponectin

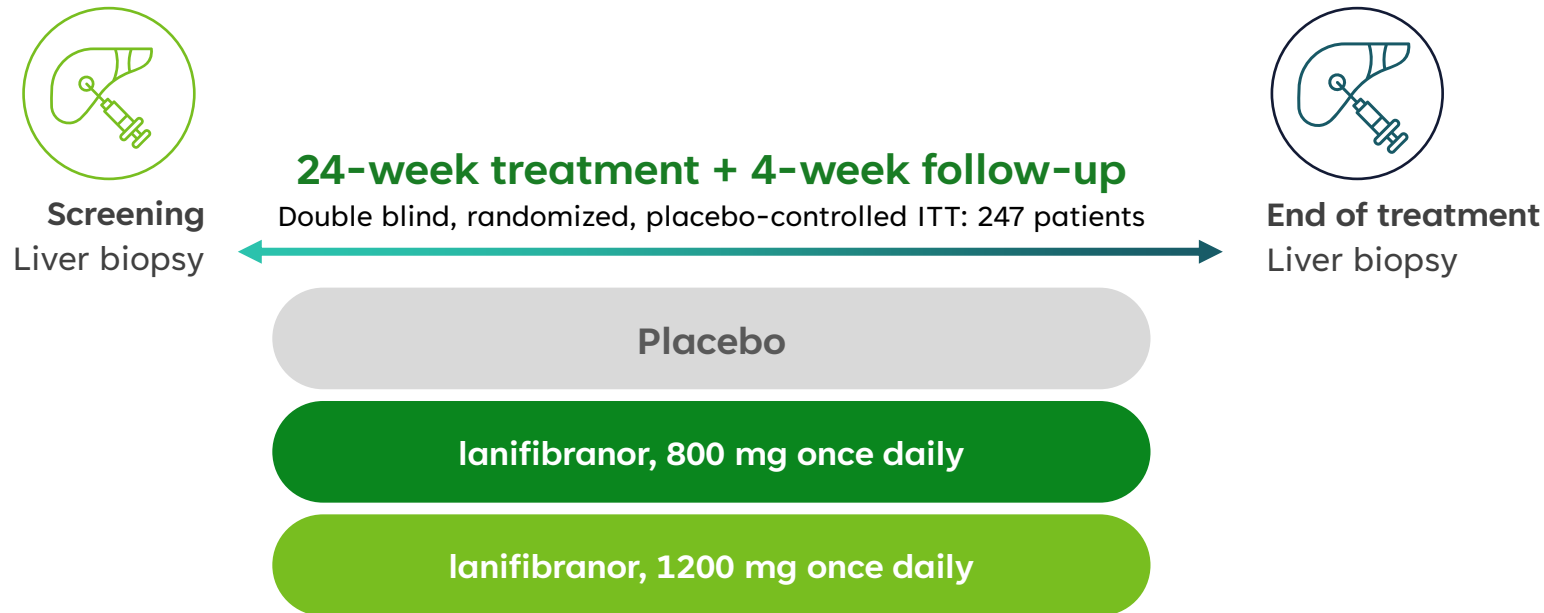
Clinical Milestones

Phase 2b NATIVE trial showed compelling data;

Phase 3 NATiV3 now underway, with topline results expected in Q4 2026

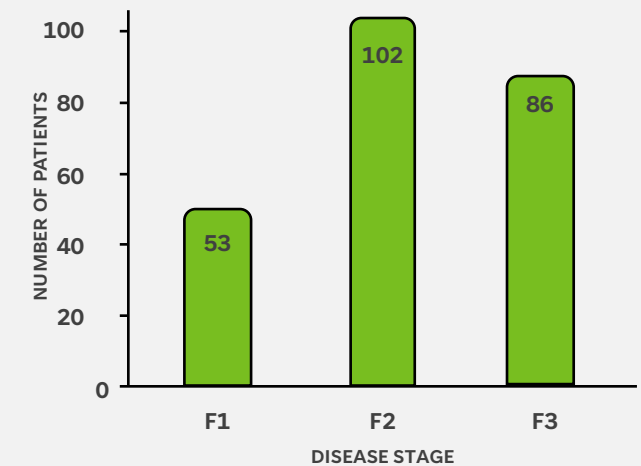
Phase 2b NATIVE Trial Design

Once-daily oral dosing; two doses, 800mg and 1200mg



Main Inclusion Criteria

Patients with biopsy-proven MASH confirmed to have Steatosis-Activity-Fibrosis (SAF) scores of 1-3 for steatosis, 3-4 for activity, and <4 for fibrosis



1:1:1 randomization

Stratification on type 2 diabetes

NATIVE Phase 2b Trial Results Support Antifibrotic Activity With Broader Metabolic Impact

Histologic improvement observed alongside favorable cardiometabolic markers at 24 weeks

18%

effect size on
improvement in fibrosis
with no worsening of
MASH versus placebo

24%

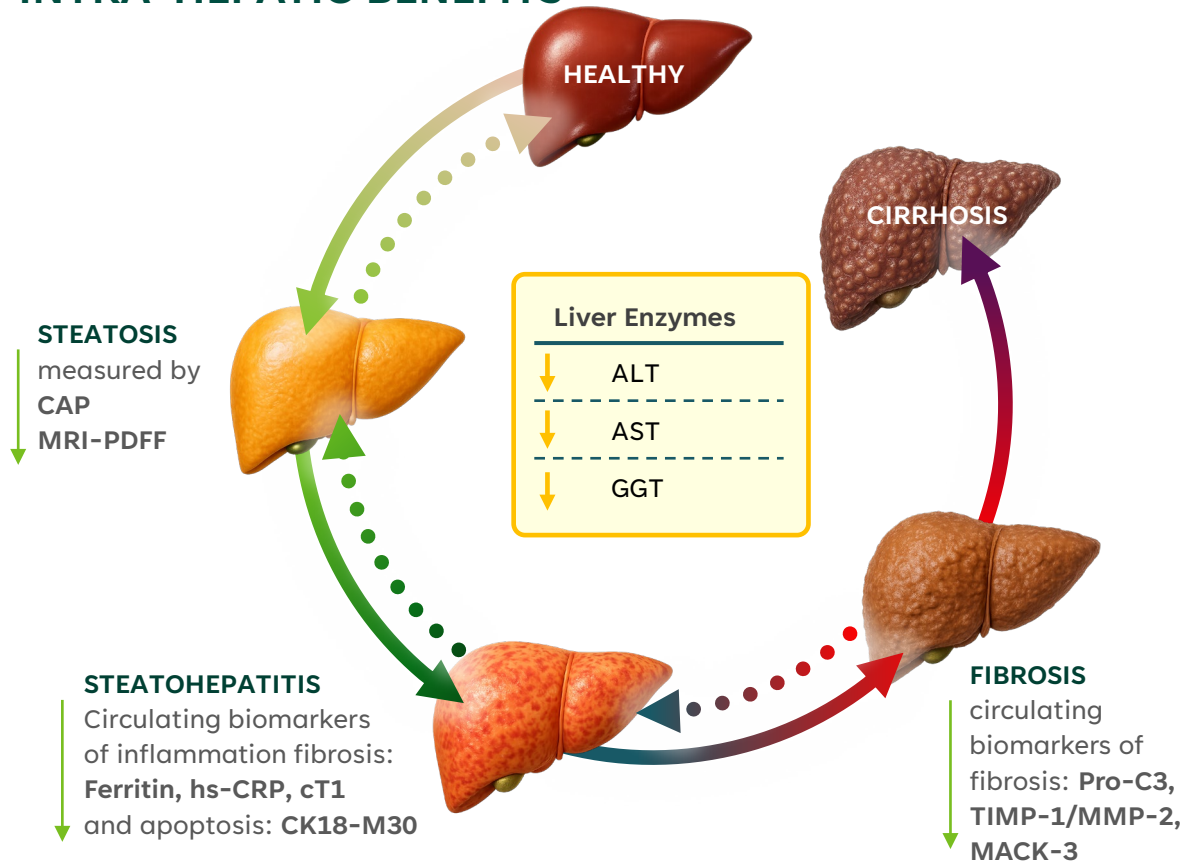
effect size on the
dual endpoint of
fibrosis improvement
and **MASH resolution**

**24
weeks**

Histologic improvement observed
alongside improved glycemic, and
cardiometabolic markers

Histologic Improvement Supported By Broad Biomarker Evidence Across Metabolic, Inflammatory And Fibrotic Pathways

INTRA-HEPATIC BENEFITS



EXTRA-HEPATIC BENEFITS

Glucose Metabolism Markers Improves insulin sensitivity and glycemic control

- ↓ Fasting Glucose
- ↓ Fasting Insulin
- ↓ Hba1c
- ↓ Fasting HOMA Index
- ↓ Hepatic Insulin Resistance
- ↑ Endogenous Glucose Production

Improves markers of
glucose metabolism in
patients with prediabetes

Cardiovascular Risk Markers Improves cardiovascular risk and lipids metabolism

- ↑ HDL-C
- ↓ Triglycerides levels
- LDL-cholesterol level
- ↓ APO-B
- ↓ APO-B/APO-A1
- ↓ APO-C3
- ↓ DBP

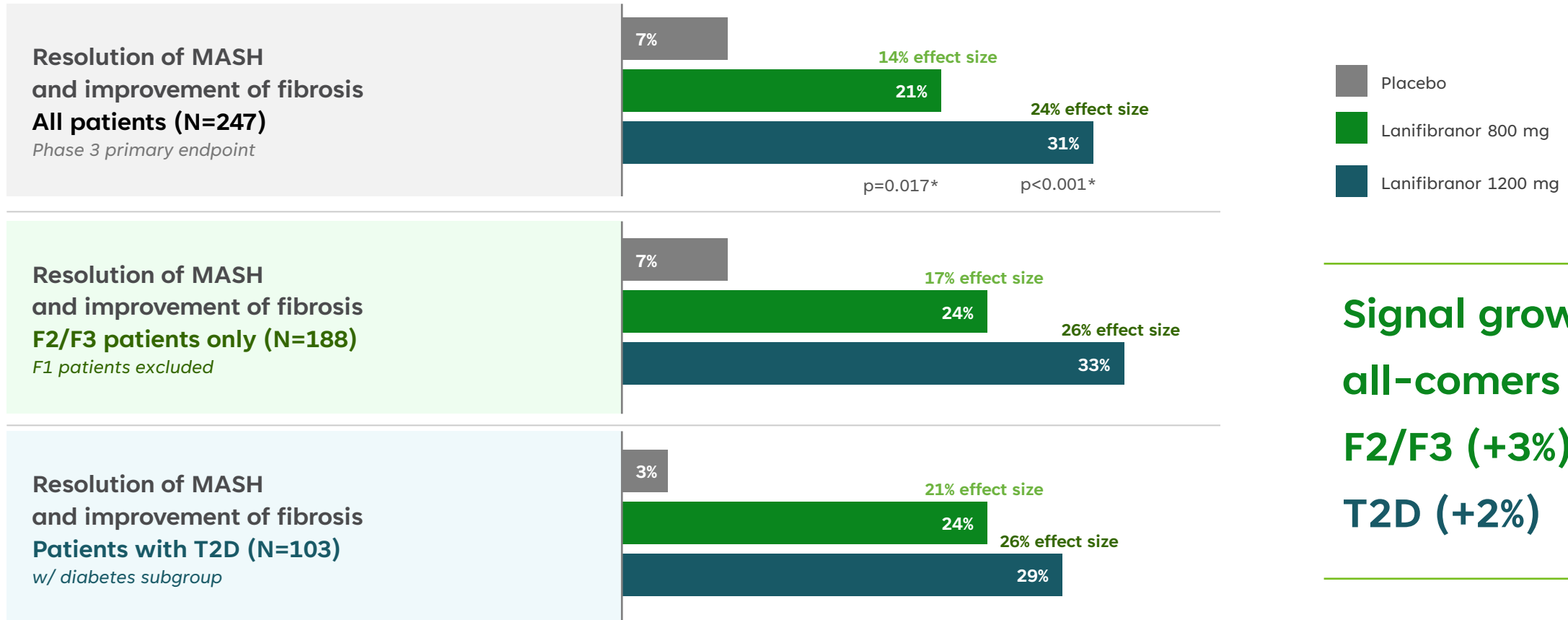
Metabolic and Anti-fibrotic Mediator

- ↑ Adiponectin (4.5x)

Lanifibranor Demonstrates Efficacy Across Early and Late-stage Disease, With And Without Diabetes



Dual endpoint signal strengthens in F2/F3 patients and in patients with T2D vs. all-comers

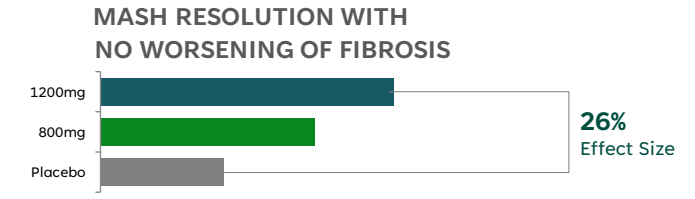
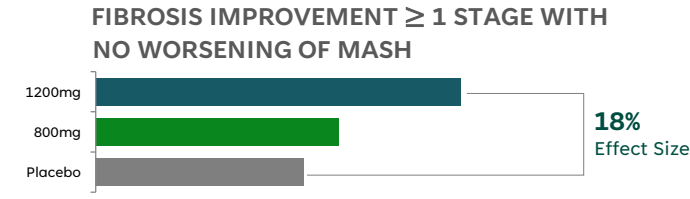
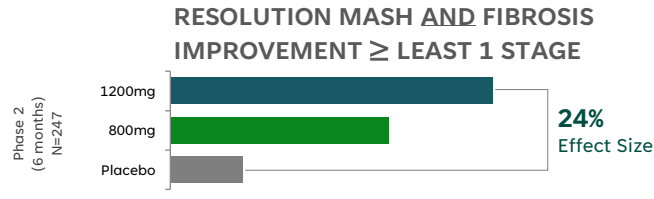


**Signal grows vs.
all-comers in:
F2/F3 (+3%)
T2D (+2%)**

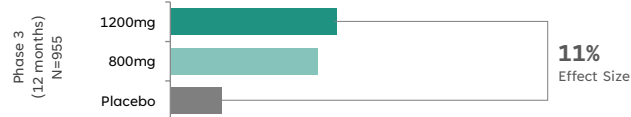
Comparison of Lanifibranor with Other Oral and Injectable Compounds

ORAL

inventiva
LANIFIBRANOR
(Pan-PPAR ($\alpha/\gamma/\delta$))

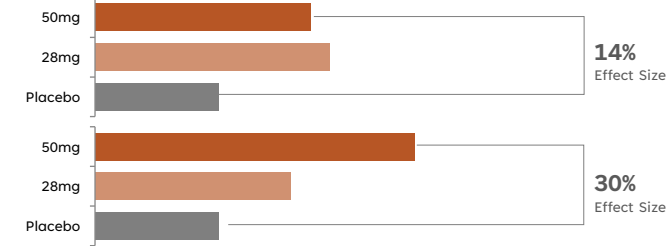
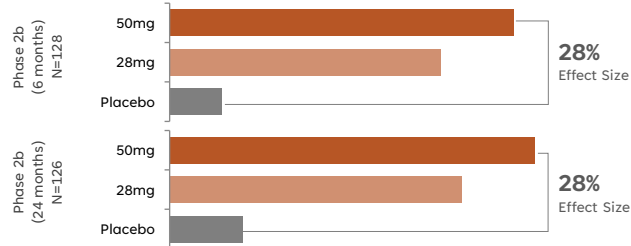


Madrigal
RESMETIROM
(THR- β)

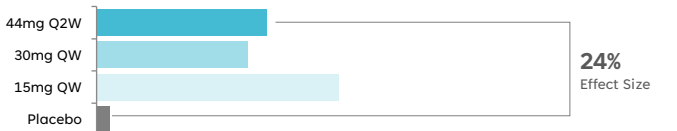
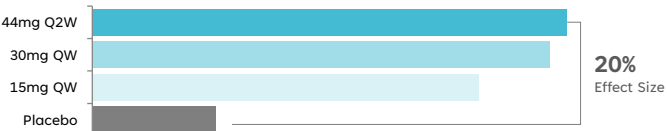
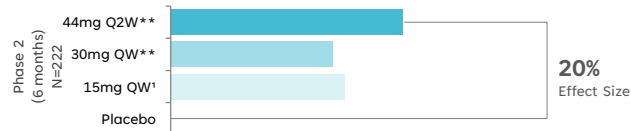


INJECTABLE

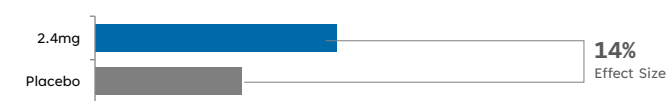
novo nordisk
akero
EFRUXIFERMIN
(FGF-21)



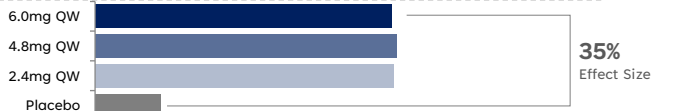
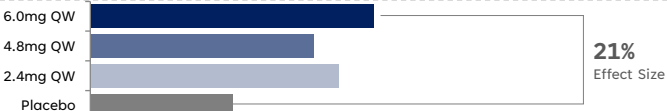
Roche **89bio**
PEGOZAFERMIN
(FGF-21)



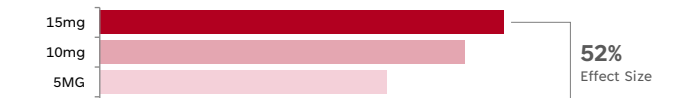
novo nordisk
SEMAGLUTIDE
(GLP1-RA)



Boehringer Ingelheim
SURVODUTIDE
(GLP1-RA Dual Agonists)



Lilly
TIRZEPATIDE
(GLP1-RA Dual Agonists)



**Resmetirom and semaglutide have been approved under accelerated approval by the FDA, and Resmetirom has been approved under conditional approval by the EMA. Source: lanifibranor native results; Efruxifermin Safety and efficacy of once-weekly efruxifermin versus placebo in non-alcoholic steatohepatitis (HARMONY): a multicentre, randomised, double-blind, placebo-controlled, Phase 2b trial. Lancet Gastroenterology October 2023 ; Semaglutide Phase 3 ESSENCE trial of semaglutide 2.4mg in participants with non-cirrhotic non-alcoholic steatohepatitis; Newsome et al.; Resmetirom MAESTRO MASH top-line results webcast Dec. 19 2022, pg 10 and EASL 2023 presentation pg. 8; Efruxifermin EASL 2023 presentation pg. 8, corporate presentation of March 2024 pg 22; Survodutide A Phase 2 randomized trial for Survodutide in MASH and fibrosis, The NEJM DOI: 10.1056/NEJMoa2401755; Tirzepatide Tirzepatide for Metabolic Dysfunction-Associated Steatohepatitis with Liver Fibrosis, The NEJM DOI: 10.1056/NEJMoa2401943



NATIVE Safety and Tolerability Profile

Investigator-reported most-frequent adverse events¹

	Placebo (N = 81)	800 mg (N = 83)	1200 mg (N = 83)
Diarrhea	1 (1%)	8 (10%)	10 (12%)
Fatigue [§]	8 (10%)	3 (4%)	11 (13%)
Nausea	3 (4%)	8 (10%)	7 (8%)
Weight Gain	-	8 (10%)	7 (8%)
Peripheral Edema ^{**}	2 (2.5%)	5 (6%)	7* (8%)
Drug-related Peripheral Edema	-	2 (2%)	2 (2%)
Headache	4 (5%)	4 (5%)	7 (8%)
Abdominal Pain	4 (5%)	4 (5%)	5 (6%)
Dizziness	3 (4%)	2 (2%)	6 (7%)
Anemia [‡]	-	1 (1%)	6 (7%)
Constipation	6 (7%)	3 (4%)	5 (6%)
Increase In Aminotransferase Level [¶]	1 (1%)	5 (6%)	3 (4%)

- ~50% of treated patients had **no weight gain**
- Where weight gain occurred (~30% had >5%), **fewer than 10% were flagged as AEs** by investigators
- Weight gain coincided with **broad metabolic improvement**:
 - ✓ Adiponectin
 - ✓ Lipids
 - ✓ Liver Enzymes
 - ✓ Liver Histology

Lanifibranor was intentionally designed with low potency and balanced PPAR- $\alpha/\delta/\gamma$ activation to mitigate class-related safety liabilities.

Early clinical data are consistent with the safety design hypothesis.

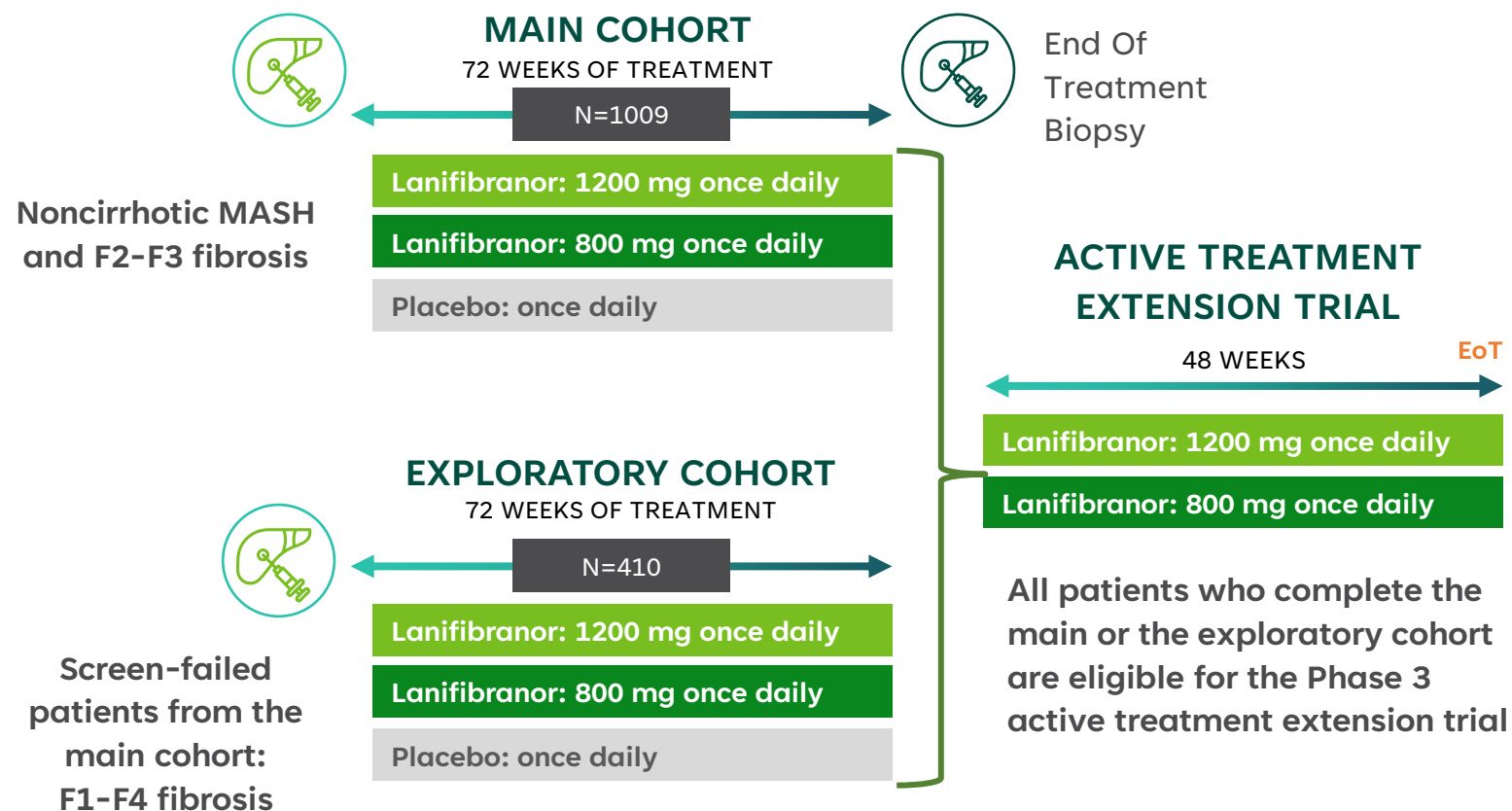
¶ Adverse events are the ones reported by investigators and include those that occurred in more than 5% of patients in either lanifibranor group. § Fatigue included asthenia. || Abdominal pain included upper and lower abdominal pain.

** Peripheral edema (bilateral ankle edema): usually mild, in most cases no treatment was required, a few patients received diuretics. 4 cases were considered study drug related by the investigator (2 at 800 and 1200 mg each). One case of severe intensity, which resolved by stopping treatment (lanifibranor 1200mg) for 12 days, without reoccurrence when the study treatment was resumed. All were female patients. ‡ Anemia included iron deficiency anemia and decrease hemoglobin level.

¶ Increase in aminotransferase level included increased in ALT, AST, or abnormal liver function test result.

NATiV3 Design Anchored in NATiVE Outcomes

Dose, population, and endpoints aligned with NATiVE to preserve observed treatment effect.



NATiV3 CLINICAL TRIAL

Primary Endpoint

Composite end point of patients having both MASH resolution and 1 stage of fibrosis improvement

Key Secondary Endpoints

MASH resolution and no worsening of fibrosis, fibrosis improvement, and no worsening of MASH

GLP-1 Inclusion

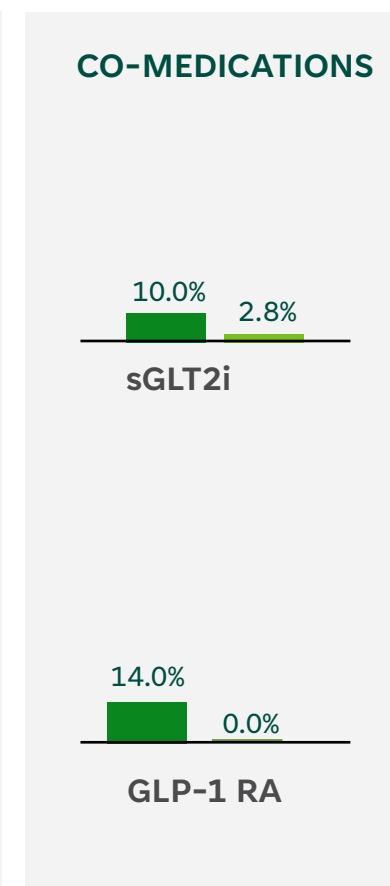
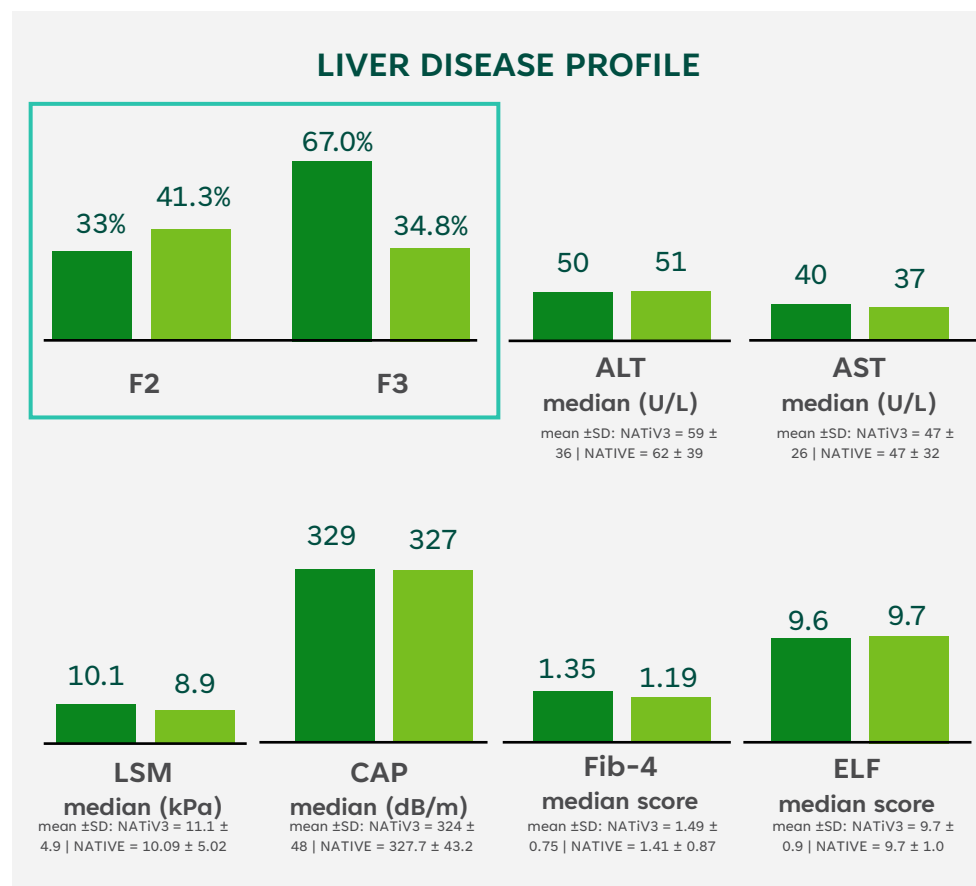
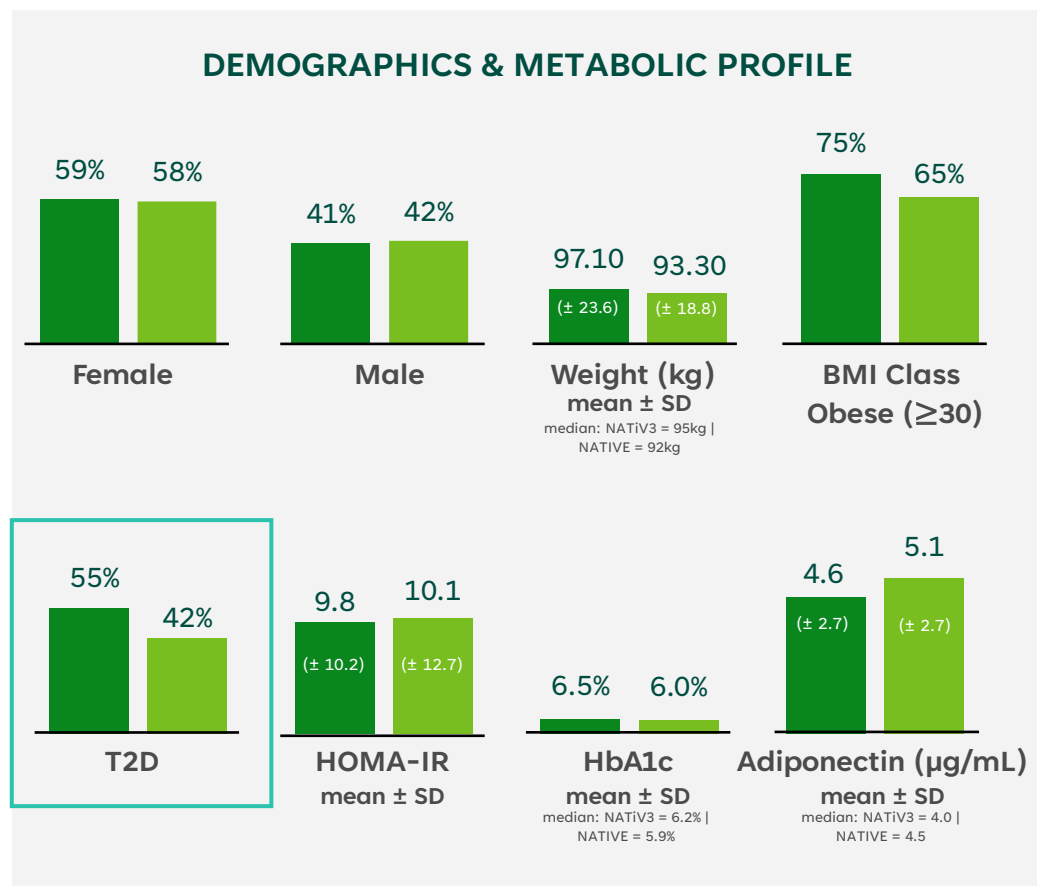
- Patients under a stable dose of GLP-1-RA for at least 3 months prior to screening can be included
- Patients can initiate a GLP-1-R after randomization into the study

Statistical Powering

- 90% considered for sample size calculations
- Stratification by fibrosis stage and diabetic status
- NATiV3 fully recruited

NATiV3 Population Consistent With NATiVE, Reflecting Real-world MASH

Higher diabetes prevalence and more advanced fibrosis



cACLD Due to MASH

Strong mechanistic rationale in a high unmet-need indication

Confirming Potential Clinical Benefit In Advanced Chronic Liver Disease Due To MASH



Planned Regulatory Filing

Intended filing for Accelerated Approval (FDA) and Conditional Approval (EMA), subject to positive Phase 3 histology results



Anticipated Confirmatory Trial

Outcomes study in patients with compensated advanced chronic liver disease due to MASH designed to confirm clinical benefits:

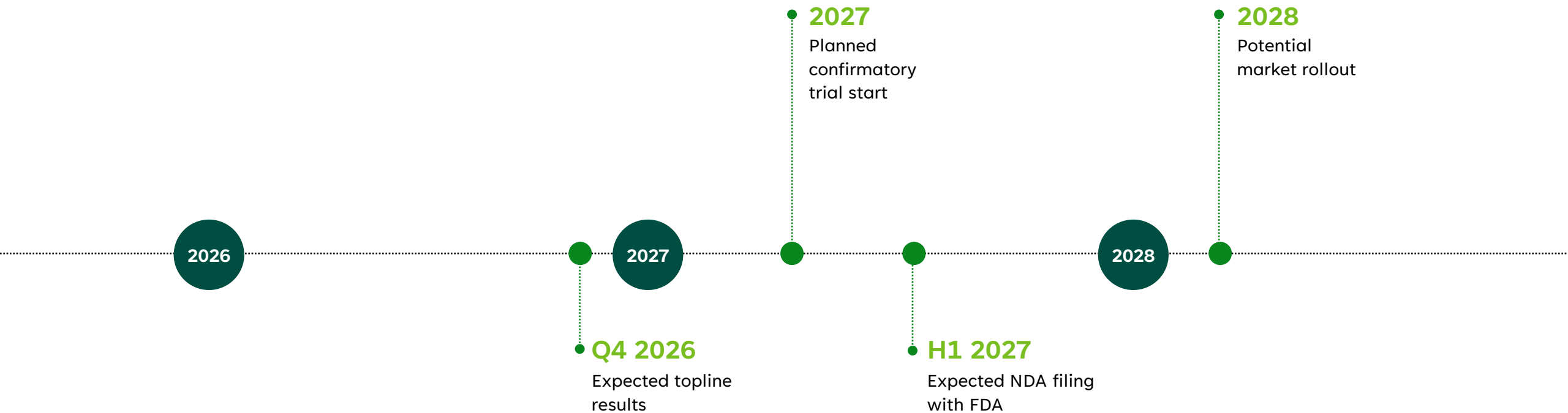
- Prevention of progression of the underlying liver disease physiology
- Prevention or reduction of associated clinical outcomes: risk of decompensation, liver transplant, death

Preclinical and early clinical signals suggest lanifibranor may provide a disease-modifying oral option for patients with cACLD due to MASH, a high unmet need

Conclusion

Topline Data Expected in Q4 2026, NDA Filing Expected in H1 2027

Lanifibranor has the potential to be the second oral liver-directed agent for the treatment of MASH if approved



Cash Runway Projected Beyond Phase 3 Readout

Q4 2025	Follow-on Raise of €149M ¹
Q1 2026	Cash balance of €176.5M ²
Q2 2026	New debt facility of up to €150M ³ with Blackrock and Claret, to replace the EIB loan facility and priced equity raise of ~€95.3M (anticipated net proceeds)
Q4 2026	Potential exercise of 3rd Tranche of Structured Financing of €116M post Topline Readout

Cash runway into early Q1 2028, including: Equity Offering, EIB Transaction, T3 Fully Exercised & Debt Financing⁴

SHARES OUTSTANDING

Pre-Transactions Fully Diluted Share Count⁵ 324M fully diluted, including 209M ordinary shares

Post-Transactions Including Equity Offering, EIB Transactions, T3 Fully Exercised & Debt Financing⁶ 430M fully diluted, including 236 ordinary shares

ROYALTY CERTIFICATES

Commences Upon Potential Launch 2% capped at €92M, 15-year term

3%, 14-year term

STRATEGIC PARTNERSHIPS

CTTQ Greater China \$265 million in additional potential milestones. Low-single-digit royalty rate.

Hepalys Japan, South Korea \$231 million in additional potential milestones. Mid-double-digit to low-20s tiered royalty rate.

(1) Gross proceeds (2) Consisting of Cash and cash equivalents of €75 million, and €121.5 million in short-term deposits. (3) New debt facility signed, subject to closing, for up to €130 million in committed tranches, subject to conditions, plus an additional uncommitted tranche of up to €20 million. (4) These estimates are based on the Company's current business plan and assume the successful closing of the Equity Offering, the completion of the EIB Transactions and the issuance of Tranches A, B and C of the Debt Financing Transaction, and the exercise in full of the Tranche 3 warrants previously issued by the Company in the Structured Financing for potential proceeds of up to €116.0 million, and exclude any potential milestones payable to or by the Company and any additional expenditures related to the product candidate or resulting from the potential in licensing or acquisition of additional product candidates or technologies, or any associated development the Company may pursue. The Company may have based these estimates on assumptions that are incorrect, and the Company may end up using its resources sooner than anticipated. These estimates may be shortened in the event of an increase, in expenditure relating to the development programs beyond the Company's expectations, or if the development program progresses more quickly than expected. There can be no assurance whether, and to what extent, the Tranche 3 warrants will be exercised, if at all. (5) Excluding T3 fully diluted shares (77.3M). (6) Reflects the share capital of the Company following completion of the Equity Offering, the EIB Transactions, the issuance of Tranches A, B and C of the Debt Financing Transaction, and exercise in full of the Tranche 3 warrants previously issued by the Company in the Structured Financing.

Pioneering the Future of MASH



Novel investigational pan-PPAR agonist engages the core drivers of fibrosis progression



Phase 2b showed **24%** effect size on **improvement of fibrosis and MASH resolution** versus placebo after 6 months



Phase 2b showed **18%** effect size on **improvement of fibrosis with no worsening of MASH** versus placebo after 6 months



Phase 3 topline data readout anticipated in **Q4 2026**

2026

Executive Priorities:



Topline results of NATiV3 Phase 3 expected in Q4 2026 assessing endpoint: Fibrosis improvement and MASH resolution



Regulatory filings preparation, ready to move swiftly if positive results



Launch strategy readiness



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